3BROTHERS

ONE HOSPITAL, TWO UTILIZATION SYSTEMS

PRESENTATION

The hospital counts appointments. But 10% of its patients don't have appointments. They have subscriptions

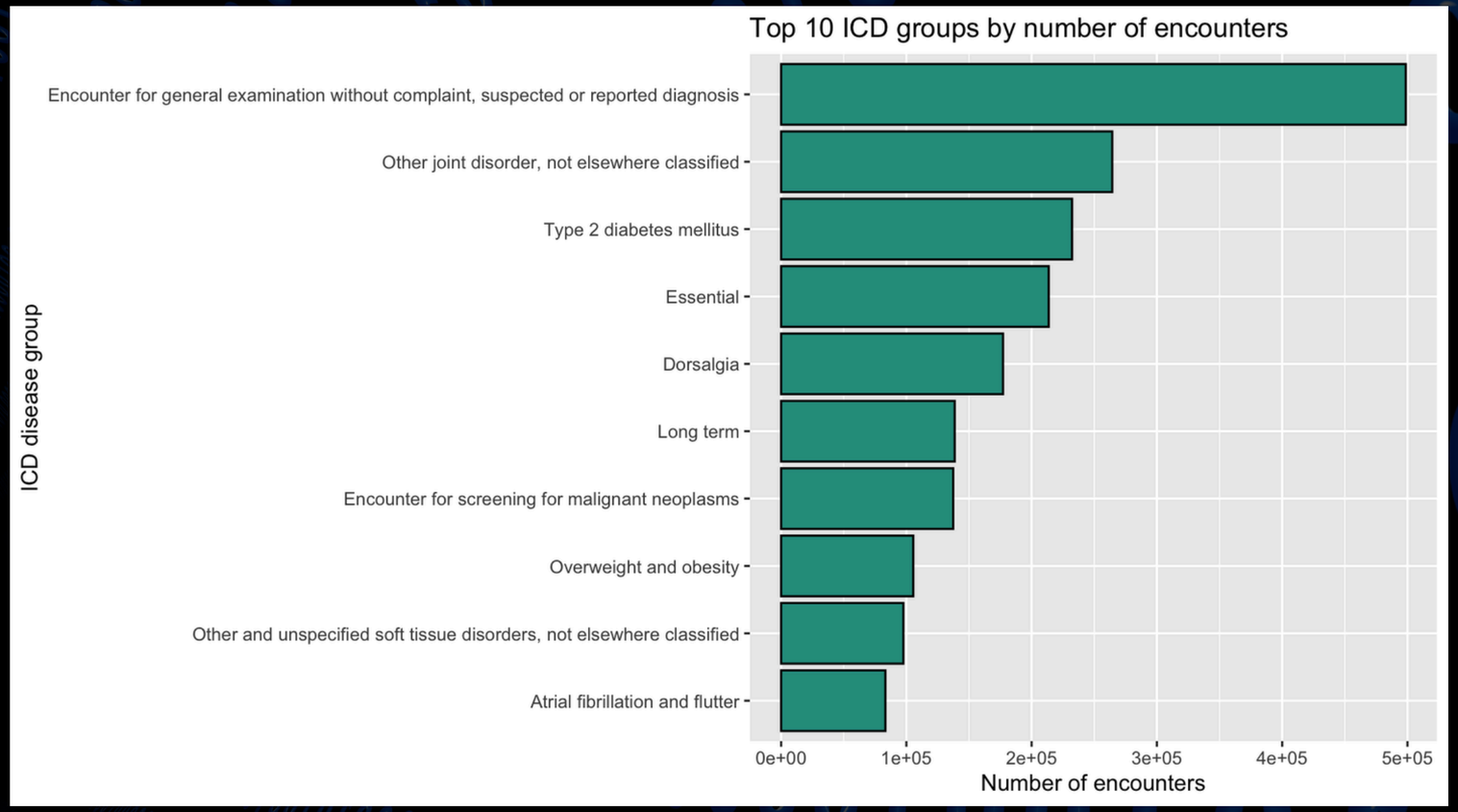
7.6M ENCOUNTERS | 947K PATIENTS | 2022–2025

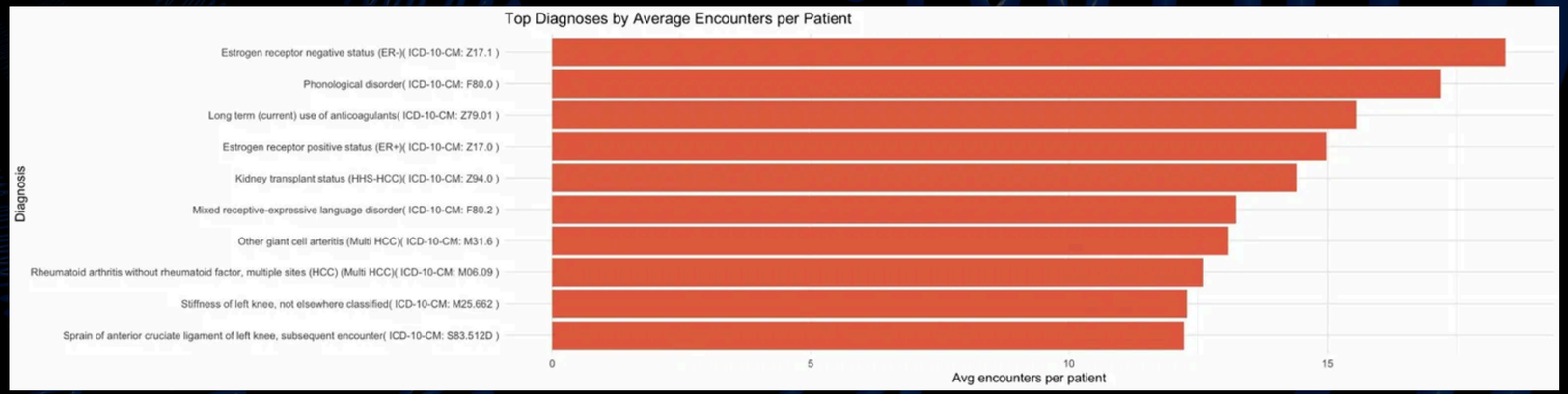


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Routine volume hides concentrated care intensity

- Total encounter volume is dominated by routine and preventive care
- The hospital appears to be driven by broad, low-complexity demand
- But average encounters per patient tell a different story
- Chronic and complex conditions generate much higher repeat intensity
- top 10% each visit SVH 55+ times in 4 years, roughly monthly contact



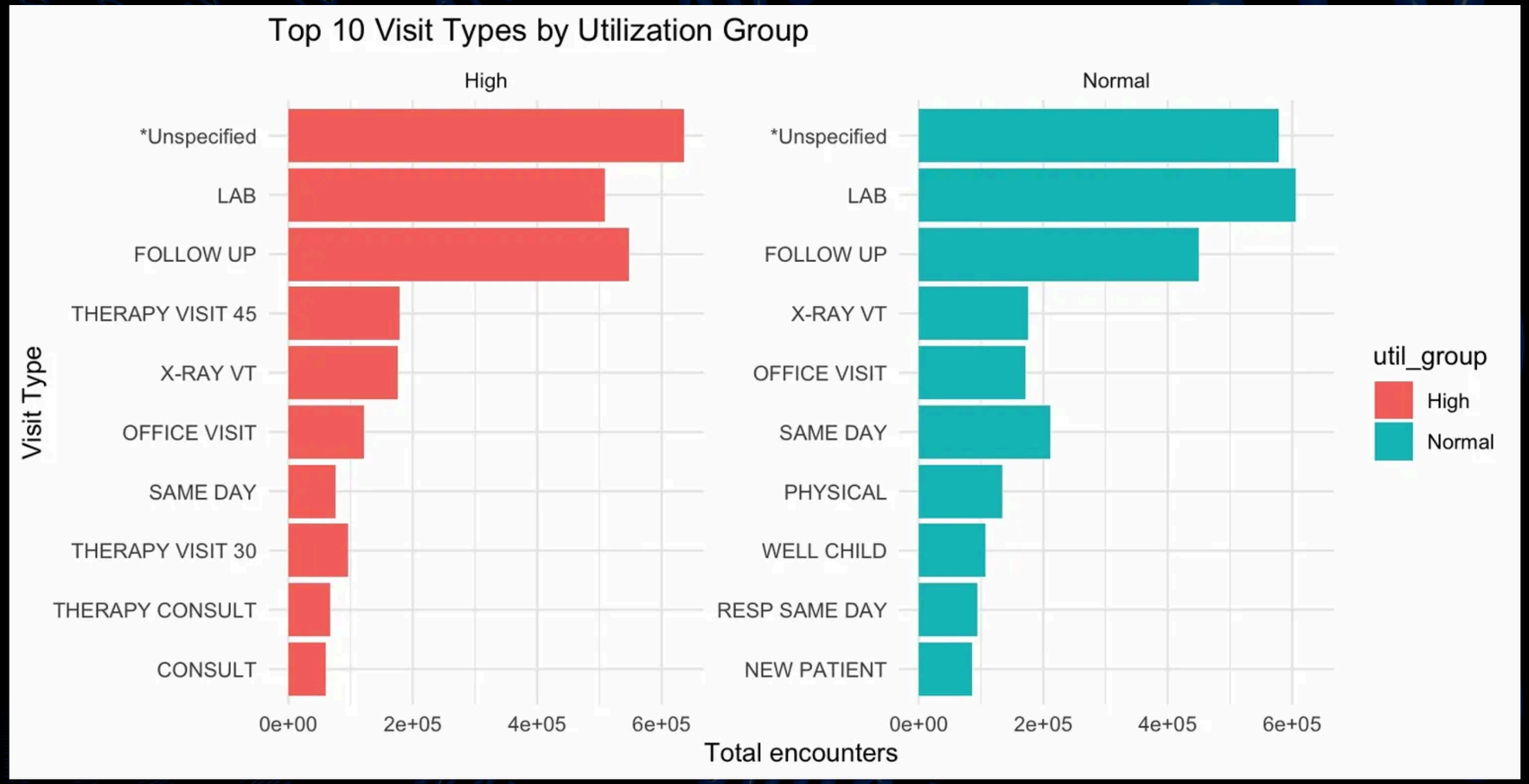


After removing Blood & Cancer groups (C & D)

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High utilizers move through structured care cycles

- Normal patients are concentrated in office visits, new patient visits, and routine check-ups
- High utilizers are concentrated in lab, follow-up, and therapy visits
- This pattern suggests repeated utilization is structured, not random
- High use reflects ongoing care pathways rather than isolated return visits



**High Utilization Group = Top 10 Percentile (> 55 visit)*

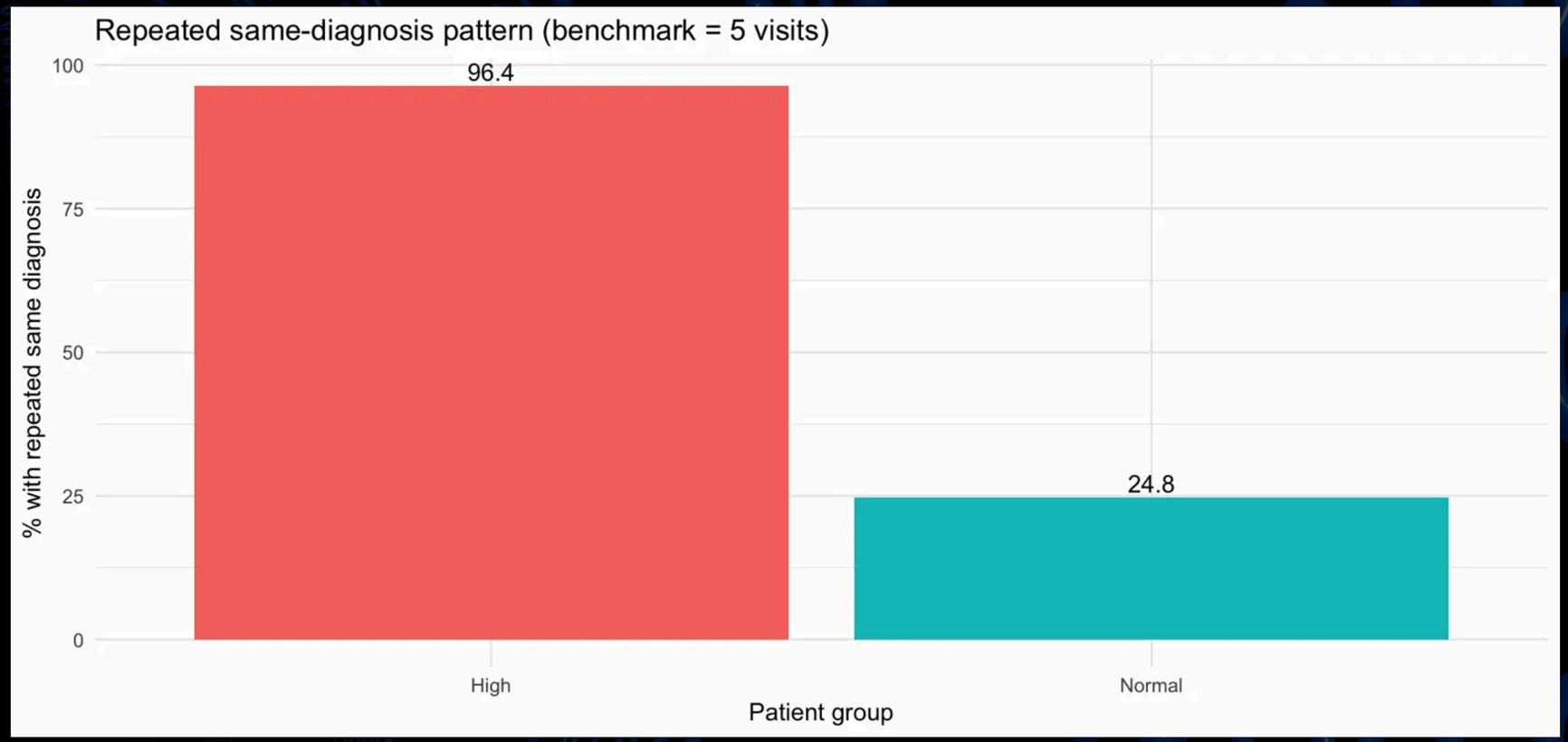
Chi-Square Test of Independence: χ^2 = 860,277 and p < 0.001

→ ***Very strong relationship between Visit Type and Ultilization Group***

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Repetition is the clearest marker of high utilization

- Around 96 percent of high utilizers repeat the same diagnosis at least 5 times
- Only around 25 percent of normal patients show the same pattern
- The difference reveals a clear separation between two patient groups
- Repetition here signals concentrated chronic care demand, not just excess use



Benchmark: 5 or more visits for the same diagnosis

Two-proportion z-test:

High utilizers: 96.4% Normal patients: 24.8%

Difference: 71.6 percentage points 95% Confidence Interval: 70.6% to 72.6%

z = 145.66

p-value < 0.001

→ ***This is a structural gap in healthcare demand***

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The hospital thinks it runs on appointments. It actually runs on subscriptions

- The hospital is serving both routine care demand and high-intensity chronic care demand
- But routine activity more visible than chronic repeat burden

Our Recommendations

1. Dual-metric dashboard

Track volume AND per-patient intensity side by side.

The 10% driving chronic complexity is invisible when only volume is reported.

2. Early subscription

Flag patients hitting 3 same-diagnosis visits, two steps before the 5-visit threshold.

Route to care coordination early.

3. Staff with reality

Our evidence shows high utilizers concentrate in lab, follow-up, and therapy.

Capacity planning should match this concentrated demand, not the broad routine volume that masks it.